

Heritage Valley Beaver

1000 Dutch Ridge Rd, Beaver, PA 15009

DISCHARGE SUMMARY Document #: DIS-020

Patient & Admission

Name	Evan Ashley
Date of Birth	1975-07-14
Phone	878-555-7527
Address	148 Eric Track Beaver, PA, 15009
MRN	MRN-543039
Admit Date	2026-04-27
Discharge Date	2026-05-03
Attending	Erin Davis, MD
Attending NPI	8044369956

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	HSB784 801845

Primary Discharge Diagnosis

Atrial flutter s/p cavotricuspid isthmus ablation

ICD-10: I48.1

Hospital Course

The patient presented with symptomatic atrial flutter characterized by palpitations, dyspnea on exertion, and presyncope. Baseline electrocardiography demonstrated a regular atrial flutter with 2:1 atrioventricular conduction and a ventricular rate in the 140s. Transthoracic echocardiography revealed normal left ventricular ejection fraction and no structural heart disease. The patient had failed rate control on metoprolol and was intolerant of amiodarone due to gastrointestinal side effects, making her an appropriate candidate for catheter ablation. She was admitted for elective cavotricuspid isthmus ablation with the goal of achieving bidirectional block and eliminating the arrhythmia substrate.

The ablation procedure was performed on hospital day two without complications. Electrophysiology mapping confirmed typical atrial flutter with counterclockwise reentry. Radiofrequency ablation was successfully delivered across the cavotricuspid isthmus, and bidirectional conduction block was demonstrated. The patient tolerated the procedure well and remained hemodynamically stable throughout. Post-procedure electrocardiography showed normal sinus rhythm. She was monitored on telemetry for the remainder of her stay with no recurrence of atrial flutter or other arrhythmias. Metoprolol was continued for rate control and blood pressure management.

The patient's recovery was uncomplicated. She remained ambulating and asymptomatic, with no chest pain, dyspnea, or hemodynamic instability. Groin access sites were without hematoma or bleeding. She was discharged on hospital day six in stable condition with close outpatient follow-up arranged with cardiology and electrophysiology. She was counseled on activity restrictions, wound care, and signs of recurrent arrhythmia. Metoprolol was continued at the current dose with instructions to follow up in one week for reassessment and consideration of rate-control medication adjustment.

Procedures Performed

Cavotricuspid isthmus ablation	CPT 93653
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Medication Changes

Metoprolol tartrate 25 mg BID	ADJUSTED
Flecainide 100 mg BID	STARTED
Amiodarone	STOPPED
Apixaban 5 mg BID	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	21 days
Urgency Tier	MEDIUM
Clinical Flags	post-ablation anticoagulation required

Dictated and electronically signed by:
Erin Davis, MD
NPI: 8044369956
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